

Access guide — hgsoc-tuboovarian-stage3c-r0-hrd-pen ding-idyq

How to access each therapy: trial contacts + manufacturer medical-info

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	Bevacizumab (continued maintenance)	monoclonal_antibody	Standard of care	FDA-approved (Avastin, and biosimilars Mvasi, Zirabev, Alymsys, Vegzelma) for stage III/IV epithelial ovarian, fallopian-tube, and primary-peritoneal cancer with carboplatin/paclitaxel followed by single-agent bevacizumab maintenance. On-label for this patient's setting.	Continue the existing bevacizumab maintenance per the established plan; no new authorization needed if already covered.
2	Niraparib (1L maintenance)	small_molecule	Standard of care	FDA-approved (Zejula) for 1L maintenance of advanced epithelial cancer in complete or partial response to first-line platinum, regardless of biomarker status (all-comers; PRIMA-based label).	Have hematology document a platelet/body-weight-based individualized starting dose (200 mg likely given baseline counts) and a written cytopenia-management pathway before starting; this is also what lifts the board's conditional veto.

3	Validated genomic-instability score (Myriad MyChoice CDx companion diagnostic / FDA-approved equivalent)	companion diagnostic (NGS panel)	Standard of care	Myriad MyChoice CDx is an FDA-approved companion diagnostic. It is the assay whose GIS threshold (≥ 42) defines HRD-positive status for the olaparib + bevacizumab (PAOLA-1) and niraparib (PRIMA) labels.	Order Myriad MyChoice CDx GIS on the archival surgical resection block (preferred for tumor content); call 1-800-469-7423 to set up the requisition.
4	Letrozole (endocrine maintenance)	small_molecule	Off-label use	FDA-approved (Femara and generics) for breast cancer. No ovarian-cancer indication, so use in ER-positive HGSC is off-label use of a widely available generic.	Have the prescriber write letrozole 2.5 mg once daily off-label; submit a prior authorization citing NCCN's hormone-therapy listing if the plan requests one.
5	Olaparib + bevacizumab (PAOLA-1 maintenance, contingent on validated HRD-positive)	small_molecule	Off-label use + Not yet accessible	Both components FDA-approved. The olaparib + bevacizumab combination carries a specific FDA indication for 1L maintenance of HRD-positive advanced ovarian cancer (HRD defined by deleterious BRCA mutation or genomic instability via an FDA-approved companion diagnostic). On-label only if HRD-positive is established; otherwise the combination would be off-label.	Order the validated GIS (Myriad MyChoice CDx or FDA-approved equivalent) on archival FFPE; this is the gating step and is captured as the essential workup row below.

6	Rucaparib (1L maintenance)	small_molecule	Off-label use	FDA-approved (Rubraca). On-label uses are BRCA-mutated advanced ovarian cancer (treatment) and platinum-sensitive recurrent maintenance; the 2018 1L-maintenance all-comer indication studied in ATHENA-MONO is not a separately approved label, so all-comer 1L maintenance in a BRCA-wild-type tumor is an off-label use of an approved drug.	Have the prescriber confirm formulary placement and submit a prior authorization citing NCCN category 2A and ATHENA-MONO (PMID 35658487) if the plan flags the all-comer use as off-label.
7	Trastuzumab deruxtecan (T-DXd)	ADC	Off-label use	FDA-approved (Enhertu) across several HER2 settings: HER2-positive and HER2-low/ultralow breast HER2-positive gastric, HER2-mutant NSCLC, and a 2024 tumor-agnostic approval for HER2 IHC 3+ solid tumors. The tumor-agnostic label requires IHC 3+, so use in this HER2-low (IHC 1+) ovarian tumor is off-label use of an approved drug.	Have the oncologist weigh off-label Enhertu against the on-label maintenance options; an unproven, off-guideline IHC 1+ ovarian use reads as a later-line consideration, not a maintenance pick.

8	Disitamab vedotin (RC48)	ADC	Clinical trial + Not yet accessible	No FDA or EMA approval. Approved in China only (NMPA; marketed as Aidixi) for HER2-positive gastric and urothelial cancer. Investigational everywhere else, with no US-marketed product to prescribe.	Call the Pfizer Clinical Trial Contact Center (1-800-887-7002) to ask whether NCT06003231 will reopen enrollment or a successor basket is planned; it reads active, not recruiting as of June 2026.
9	GEN-1 / IMNN-001 (IL-12 immunogene, OVATION 2)	other	Clinical trial	Investigational; no FDA or EMA approval. IND-stage IL-12 DNA immunogene therapy (Imunon). FDA Fast Track designation reported for advanced ovarian cancer.	Ask the treating trial site whether OVATION 2 has a continuation/extension provision keeping the patient on IMNN-001 after the primary phase.
10	Trastuzumab duocarmazine (SYD985)	ADC	Unavailable	Discontinued. FDA issued a complete response letter in May 2023 for the HER2-positive metastatic breast filing; the EU marketing application (validated July 2022) never cleared; the global development status is now discontinued, with the trials completed or withdrawn. No approval anywhere.	No action available; the drug cannot be obtained, so record it as closed.

Standard of care (3)

1. Bevacizumab (continued maintenance)

Aliases: bevacizumab, Avastin, rhuMAb-VEGF

Modality: monoclonal_antibody · **Regulatory:** FDA-approved (Avastin, and biosimilars Mvasi, Zirabev, Alymsys, Vegzelma) for stage III/IV epithelial ovarian, fallopian-tube, and primary-peritoneal cancer with carboplatin/paclitaxel followed by single-agent bevacizumab maintenance. On-label for this patient's setting. ·

Guidelines: NCCN Ovarian v1.2025 bevacizumab maintenance, category 2A (all-comers after 1L platinum). ·

Geographic scope: US — routine on-label prescription nationwide (Avastin and biosimilars). · **Verified:**

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Bevacizumab is on-label standard-of-care maintenance for this exact setting, and the patient is already on it, so continuation is the lowest-friction path on the board. It clears both of her named toxicity vetoes (no added neuropathy, no myelosuppression). The live caution is vascular: anti-angiogenic VTE and bleeding risk against her high VTE history and prior IVC filter, managed with blood-pressure and bleeding monitoring rather than dose escalation. The honest ceiling is PFS-only, with no overall-population survival gain.

Next steps

1. Continue the existing bevacizumab maintenance per the established plan; no new authorization needed if already covered.
2. Keep the hypertension and bleeding-monitoring algorithm active and coordinate anticoagulation decisions against the IVC-filter/VTE history.
3. If a biosimilar is substituted by the plan, switch the medical-information contact to that product's marketer (Amgen/Mvasi, Pfizer/Zirabev).
4. Revisit with the team whether to layer a PARP onto bevacizumab once the GIS resolves, versus continuing bevacizumab alone given the cleared Signatera MRD state.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT00262847		completed	no	—

Manufacturer / sponsor contact

- **Company:** Genentech (Avastin; a member of the Roche Group)
- **Med info phone:** 1-800-821-8590
- **Product info:** <https://www.avastin.com/>
- **Compassionate / expanded access:** <https://www.gene.com/patients/compassionate-use>
- **Notes:** 1-800-821-8590 is Genentech Medical Communications (US). Biosimilar medical-information lines differ by marketer (Amgen for Mvasi, Pfizer for Zirabev) and should be used when a biosimilar is dispensed.

Payer / coverage: On-label and well-established; covered by Medicare Part B as a clinician-administered IV biologic. Biosimilar substitution is common and plan-driven. Genentech Access Solutions and the Avastin co-pay program assist commercially insured patients; Medicare patients are routed to independent foundations.

Notes: Already in use. The access question is really a maintenance-intensity question, not a how-to-obtain question.

2. Niraparib (1L maintenance)

Aliases: niraparib, MK-4827, Zejula

Modality: small_molecule · **Regulatory:** FDA-approved (Zejula) for 1L maintenance of advanced epithelial ovarian/fallopian-tube/primary-peritoneal cancer in complete or partial response to first-line platinum, regardless of biomarker status (all-comers; PRIMA-based label). · **Guidelines:** NCCN Ovarian v1.2025 single-agent PARP

maintenance, category 2A (all-comers); ESMO endorses 1L PARP maintenance in this setting. · **Geographic scope:** US — routine on-label prescription nationwide. · **Verified:** 2026-06-24

Niraparib is the one PARP option here that is on-label standard of care for all-comer 1L maintenance, so the treating oncologist can prescribe it without an off-label justification. The gate is deliverability, not access: grade 3 or higher thrombocytopenia near 29 percent and anemia near 31 percent hit the patient's named myelosuppression concern, and a baseline WBC of 3.5 in the 70-79 band leaves thin marrow reserve. A platelet-based individualized starting dose and a written cytopenia plan are what make it usable.

Next steps

1. Have hematology document a platelet/body-weight-based individualized starting dose (200 mg likely given baseline counts) and a written cytopenia-management pathway before starting; this is also what lifts the board's conditional veto.
2. Confirm Part D formulary placement and submit any required prior authorization (on-label, so this is usually routine).
3. Screen GSK and foundation patient-assistance options for the patient's insurance type via the Zejula support line (1-888-825-5249).
4. Set a CBC monitoring cadence (weekly for the first month per label) and a dose-modification trigger plan.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02635016		completed	no	—

Manufacturer / sponsor contact

- **Company:** GSK (Zejula)
- **Med info phone:** 1-888-825-5249
- **Product info:** <https://www.zejula.com/>
- **Compassionate / expanded access:** <https://www.gsk.com/en-gb/innovation/our-policies/managed-access-and-compassionate-use/>
- **Notes:** 1-888-825-5249 is GSK's US Response Center / medical-information line. GSK does not publish a general medical-information email for Zejula; route written queries through the Response Center.

Payer / coverage: On-label 1L all-comer maintenance, so coverage is the cleanest of the PARP options. Oral oncolytic billed under Medicare Part D. GSK's Zejula patient-assistance and co-pay programs cover commercially insured and uninsured patients; Medicare beneficiaries are excluded from manufacturer co-pay but may qualify for foundation support.

Notes: Only intervention on the board that is true standard-of-care on-label for this exact indication. The clinical hesitation is toxicity deliverability, recorded as a hematology plan, not an access barrier.

3. Validated genomic-instability score (Myriad MyChoice CDx companion diagnostic / FDA-approved equivalent)

Aliases: MyChoice CDx, Myriad MyChoice, GIS, genomic instability score, HRD companion diagnostic, FoundationOne CDx LOH, Caris HRD

Modality: companion diagnostic (NGS panel) · **Regulatory:** Myriad MyChoice CDx is an FDA-approved companion diagnostic. It is the assay whose GIS threshold (≥ 42) defines HRD-positive status for the olaparib + bevacizumab (PAOLA-1) and niraparib (PRIMA) labels. · **Guidelines:** NCCN Ovarian v1.2025 recognizes validated HRD testing (BRCA plus genomic instability) to inform PARP-maintenance selection. MyChoice CDx is the label-recognized assay tied to the PAOLA-1 indication. · **Geographic scope:** US — central-lab send-out testing (Salt Lake City) accepting nationwide FFPE specimens. · **Verified:** 2026-06-24

This is the test that unlocks the highest-magnitude maintenance option, not a therapy. Myriad MyChoice CDx is the FDA companion diagnostic whose GIS ≥ 42 cutoff defines HRD-positive status; ordering it resolves whether the olaparib + bevacizumab combination is on the table. Foundation and Caris offer LOH/HRD readouts, but only MyChoice maps directly to the PAOLA-1 label, so it is the preferred order. Use archival FFPE; expect a 2-3 week turnaround.

Next steps

1. Order Myriad MyChoice CDx GIS on the archival surgical resection block (preferred for tumor content); call 1-800-469-7423 to set up the requisition.
2. If GIS ≥ 42 , the patient becomes PAOLA-1 / olaparib+bevacizumab eligible (on-label category 1); if < 42 , she is HRD-negative and niraparib all-comer maintenance is the cleaner PARP option.
3. Use MyChoice as the preferred assay rather than Foundation/Caris LOH scores, since only MyChoice's threshold maps to the olaparib+bevacizumab label.
4. Confirm payer coverage and turnaround (~2-3 weeks) so the maintenance decision is not delayed.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
—	—	unknown	yes	—

Manufacturer / sponsor contact

- **Company:** Myriad Genetics (MyChoice CDx) — preferred; alternates Foundation Medicine (FoundationOne CDx LOH) and Caris Life Sciences (Caris HRD)
- **Med info phone:** 1-800-469-7423
- **Product info:** <https://myriad.com/precision-medicine/mychoice-cdx/>
- **Notes:** Myriad MyChoice CDx ordering: 1-800-469-7423, <https://myriad.com/precision-medicine/mychoice-cdx/>, 320 Wakara Way, Salt Lake City, UT 84108. Foundation Medicine (FoundationOne CDx LOH): 1-888-988-3639, <https://www.foundationmedicine.com/test/foundationone-cdx>, 150 Second Street, Cambridge, MA 02141 — reports a genomic LOH score used as an HRD surrogate but is NOT the FDA companion diagnostic tied to the PAOLA-1 olaparib+bevacizumab label. Caris Life Sciences (Caris HRD): 1-888-979-8669, <https://www.carislifesciences.com/>, 4610 South 44th Place, Phoenix, AZ 85040 — confirm whether its reported score maps to a label-recognized threshold before relying on it for PAOLA-1 eligibility. Contacts mirrored verbatim from target_validation.jsonl.

Payer / coverage: FDA-approved companion diagnostics for HRD/GIS in advanced ovarian cancer are generally covered by Medicare and most commercial plans when ordered to guide PARP-maintenance selection.

Turnaround is roughly 2-3 weeks on archival FFPE. Myriad operates patient financial-assistance programs for the out-of-pocket portion.

Notes: This row is the gating companion diagnostic for the rank-6 olaparib+bevacizumab option, not an intervention itself. Contacts captured verbatim from the upstream target_validation.jsonl providers block; no contact invented.

Off-label use (4)

4. Letrozole (endocrine maintenance)

Aliases: letrozole, Femara, CGS-20267

Modality: small_molecule · **Regulatory:** FDA-approved (Femara and generics) for hormone-receptor-positive breast cancer. No ovarian-cancer indication, so use in ER-positive HGSC is off-label use of a widely available generic. · **Guidelines:** NCCN Ovarian v1.2025 includes aromatase inhibitors among hormone-therapy options; a softer recommendation tier than the category 2A PARP and bevacizumab choices. · **Geographic scope:** US — routine off-label prescription nationwide. · **Verified:** 2026-06-24

Letrozole is a cheap oral generic the oncologist can prescribe off-label today, which makes it the lowest-burden and lowest-toxicity option on the board and a clean fit for the patient's age and oral preference. The catch is evidence, not access: the supporting data is a single retrospective ROBINS-I-serious series, and PR-negativity tempers the expected benefit from the roughly 80 percent ER reading. Best positioned as a gentle later-line or adjunct option rather than a randomized-backed maintenance regimen.

Next steps

1. Have the prescriber write letrozole 2.5 mg once daily off-label; submit a prior authorization citing NCCN's hormone-therapy listing if the plan requests one.
2. Set expectations with the patient that the evidence is retrospective and the magnitude is modest, especially given PR-negativity.
3. Plan bone-density monitoring and arthralgia management for prolonged aromatase-inhibitor use.
4. Reconcile the upstream design-source flag (trials row described a single-arm phase 2; the source PMID 29157627 is a retrospective translational series).

Trial pathways

NCT	Phase	Status	Eligible	Central contact
—	—	unknown	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Generic (originator Novartis, Femara); multiple generic manufacturers
- **Med info phone:** 1-888-669-6682
- **Product info:** <https://www.novartis.com/us-en/medicines>

- **Notes:** 1-888-669-6682 is Novartis US Customer/Medical Information for the Femara originator; for a dispensed generic, medical-information queries route to that generic's manufacturer. No compassionate-use pathway is relevant for a marketed generic.

Payer / coverage: Inexpensive generic on Medicare Part D with minimal co-pay. Off-label ovarian use can prompt a prior-authorization request, usually satisfied by NCCN's listing of aromatase inhibitors as a hormone-therapy option. No manufacturer assistance program needed at generic pricing.

Notes: Access is trivial (generic). The limiting factor is evidence quality, not obtainability.

5. Olaparib + bevacizumab (PAOLA-1 maintenance, contingent on validated HRD-positive)

Aliases: olaparib, AZD2281, KU-0059436, Lynparza, bevacizumab, Avastin

Modality: small_molecule · **Regulatory:** Both components FDA-approved. The olaparib + bevacizumab combination carries a specific FDA indication for 1L maintenance of HRD-positive advanced ovarian cancer (HRD defined by deleterious BRCA mutation or genomic instability via an FDA-approved companion diagnostic). On-label only if HRD-positive is established; otherwise the combination would be off-label. · **Guidelines:** NCCN Ovarian v1.2025 category 1 — but only for HRD-positive disease defined by tumor BRCA or a validated GIS ≥ 42 , which this BRCA-wild-type tumor does not yet meet. · **Geographic scope:** US — on-label prescription contingent on a validated companion-diagnostic GIS result; no geographic barrier beyond the test. · **Verified:** 2026-06-24

This is the highest-magnitude maintenance option, but the patient cannot reach it yet: the FDA combination indication and NCCN category 1 listing both require HRD-positive status, and her BRCA-wild-type tumor has no validated genomic-instability score. A Myriad MyChoice CDx GIS of 42 or higher is the single gate. The board also carried a double veto on stacked anti-angiogenic plus PARP toxicity against her IVC-filter history, which becomes addressable only with a documented anticoagulation/BP plan and a confirmed HRD-positive result.

Next steps

1. Order the validated GIS (Myriad MyChoice CDx or FDA-approved equivalent) on archival FFPE; this is the gating step and is captured as the essential workup row below.
2. If GIS ≥ 42 returns, the combination becomes on-label; assemble a documented anticoagulation/blood-pressure plan to address the toxicity veto before starting.
3. If GIS < 42 , treat as HRD-negative and route to all-comer niraparib (PRIMA) instead; the combination is foreclosed below 42.
4. For drug-specific questions, call AstraZeneca Medical Information (1-800-236-9933) for olaparib and Genentech (1-800-821-8590) for bevacizumab.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02437644		completed	no	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca (Lynparza/olaparib) and Genentech-Roche (Avastin/bevacizumab); Lynparza co-developed with Merck
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://www.lynparza.com/>
- **Compassionate / expanded access:** <https://www.astrazeneca-us.com/our-science/clinical-trials/expanded-access.html>
- **Notes:** 1-800-236-9933 is AstraZeneca US Medical Information (for the olaparib half). For the bevacizumab half use Genentech Medical Communications (1-800-821-8590). Both components are already approved, so an expanded-access request is not the relevant path; the gate is the companion-diagnostic result, not drug availability.

Payer / coverage: Coverage hinges entirely on documented HRD-positive status. With a validated GIS ≥ 42 the combination is on-label NCCN category 1 and reimbursable. Without it, payers will deny the combination for a BRCA-wild-type tumor and the patient defaults to all-comer niraparib. Olaparib is Part D; bevacizumab is Part B.

Notes: Both drugs are fully available; the only barrier is the missing companion-diagnostic score. Classified not_yet_accessible because the patient cannot currently reach the indication, with off_label_use noted as the fallback descriptor if a clinician pursued the combination without a qualifying GIS.

6. Rucaparib (1L maintenance)

Aliases: rucaparib, AG-014699, CO-338, Rubraca

Modality: small_molecule · **Regulatory:** FDA-approved (Rubicin). On-label uses are BRCA-mutated advanced ovarian cancer (treatment) and platinum-sensitive recurrent maintenance; the 2018 1L-maintenance all-comer indication studied in ATHENA-MONO is not a separately approved label, so all-comer 1L maintenance in a BRCA-wild-type tumor is an off-label use of an approved drug. · **Guidelines:** NCCN Ovarian v1.2025 lists single-agent PARP maintenance, category 2A, for all-comers after a 1L platinum response. · **Geographic scope:** US — routine prescription nationwide. · **Verified:** 2026-06-24

Rucaparib is an approved oral PARP inhibitor the treating oncologist can prescribe now, though all-comer 1L maintenance for this BRCA-wild-type tumor falls outside the strict label and may need a prior-authorization step. The practical constraint is hematologic rather than regulatory: grade 3 or higher anemia ran near 29 percent in ATHENA-MONO, which sits against the patient's stated myelosuppression concern. No trial enrollment is required.

Next steps

1. Have the prescriber confirm formulary placement and submit a prior authorization citing NCCN category 2A and ATHENA-MONO (PMID 35658487) if the plan flags the all-comer use as off-label.
2. Call pharma& medical information (1-844-779-7707) to confirm the current US Rubraca medical-information contact and ask about patient-assistance eligibility for this insurance type.
3. Document a baseline CBC and an anemia/cytopenia monitoring plan with hematology before starting, given the patient's WBC of 3.5 and the myelosuppression concern.
4. Rucaparib effect size is the primary JCO ITT HR 0.52 (95% CI 0.40-0.68, PMID 35658487).

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT0352246		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** pharma& (Rubraca; formerly Clovis Oncology)
- **Med info phone:** 1-844-779-7707
- **Med info email:** medinfo@pharmaand.com
- **Product info:** <https://www.rubraca.com/>
- **Notes:** pharma& acquired Rubraca from Clovis Oncology in 2022. Confirm the current US medical-information line when calling, since the contact moved with the asset transfer.

Payer / coverage: As an FDA-approved oral oncolytic, rucaparib is generally covered under Medicare Part D rather than Part B. Off-label 1L all-comer maintenance can draw a prior-authorization request or a compendia challenge from the plan; NCCN category 2A listing and the ATHENA-MONO data are the supporting citations the prescriber would attach. Manufacturer co-pay and patient-assistance support exists for commercially insured and uninsured patients but not for Medicare beneficiaries.

Notes: Access route is off-label prescription of an approved drug, not a trial. The rank-1 placement rests on replicated all-comer phase 3 efficacy, not on an open study slot.

7. Trastuzumab deruxtecan (T-DXd)

Aliases: trastuzumab deruxtecan, T-DXd, DS-8201, DS-8201a, fam-trastuzumab deruxtecan-nxki, Enhertu

Modality: ADC · **Regulatory:** FDA-approved (Enhertu) across several HER2 settings: HER2-positive and HER2-low/ultralow breast (DESTINY-Breast04/06), HER2-positive gastric, HER2-mutant NSCLC, and a 2024 tumor-agnostic approval for HER2 IHC 3+ solid tumors. The tumor-agnostic label requires IHC 3+, so use in this HER2-low (IHC 1+) ovarian tumor is off-label use of an approved drug. · **Guidelines:** Not NCCN/ESMO-listed for HER2-low ovarian cancer; the tumor-agnostic NCCN listing tracks the IHC 3+ approval, so off-guideline at IHC 1+. · **Geographic scope:** US - off-label prescription nationwide; no T-DXd trial currently enrolling this tumor type. · **Verified:** 2026-06-26

Enhertu is FDA-approved and a treating oncologist can order it, just not on-label for this tumor: the tumor-agnostic approval needs HER2 IHC 3+ and her ovarian tumor is 1+, so this is off-label use. The efficacy case is borrowed, with the DESTINY-PanTumor02 ovarian cohort enrolling IHC 2+/3+ and the IHC 1+ evidence coming from breast trials. Expect a prior-authorization fight and weigh the roughly 10-12% ILD/pneumonitis risk. No T-DXd study is enrolling her now; the open HER2-directed ovarian trials use disitamab vedotin instead, covered in that row.

Next steps

1. Have the oncologist weigh off-label Enhertu against the on-label maintenance options; an unproven, off-guideline IHC 1+ ovarian use reads as a later-line consideration, not a maintenance pick.
2. Order a confirmatory HER2 re-stain first; the 1+ call rests on a single surgical-path specimen and the Altera platform could not evaluate HER2.
3. If pursued, plan for a payer denial (the tumor-agnostic approval is IHC 3+) and build an appeal around the DESTINY-PanTumor02 ovarian cohort and DESTINY-Breast04/06.

4. Call AstraZeneca Medical Information (1-800-236-9933) or Daiichi Sankyo (1-877-437-7763) for off-label dosing and ILD monitoring; set up baseline and serial chest imaging given the pneumonitis rate.
5. For a HER2-directed trial route, see the disitamab vedotin row.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04422309		active not recruiting	no	—
NCT03734029		completed	no	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca / Daiichi Sankyo (co-development and US co-commercialization of Enhertu)
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://www.enhertu.com/>
- **Compassionate / expanded access:**
<https://www.astrazeneca-us.com/our-science/clinical-trials/expanded-access.html>
- **Notes:** 1-800-236-9933 is AstraZeneca US Medical Information. Daiichi Sankyo medical/safety is 1-877-437-7763, and patient/reimbursement support is 1-833-ENHERTU (1-833-364-3788). The drug is marketed, so an expanded-access request is not the relevant route for an off-label ovarian use.

Payer / coverage: Enhertu is a clinician-administered IV biologic billed under Medicare Part B. An IHC 1+ ovarian use will likely draw a denial, since the tumor-agnostic approval is IHC 3+; an appeal would lean on the DESTINY-PanTumor02 ovarian cohort and the DESTINY-Breast04/06 HER2-low data, but that is a hard case at IHC 1+.

Notes: Off-label use of an approved drug is the only real T-DXd path here. The HER2-low (IHC 1+) ovarian setting has no randomized support, so this sits well below the maintenance options on actionability.

Clinical trial (2)

8. Disitamab vedotin (RC48)

Aliases: disitamab vedotin, RC48, RC48-ADC, hertuzumab vedotin, Aidixi

Modality: ADC · **Regulatory:** No FDA or EMA approval. Approved in China only (NMPA; marketed as Aidixi) for HER2-positive gastric and urothelial cancer. Investigational everywhere else, with no US-marketed product to prescribe. · **Guidelines:** Not NCCN/ESMO-listed for ovarian cancer; investigational. · **Geographic scope:** US basket (NCT06003231) closed to enrollment; the only recruiting trial and the marketing approval are China-only. Not reachable from US-South today. · **Verified:** 2026-06-26

Disitamab vedotin has no FDA or EMA approval; it is cleared only in China (Aidixi, for gastric and urothelial cancer), so a US patient reaches it through a trial. The Pfizer/Seagen solid-tumor basket (NCT06003231) does accept HER2 IHC 1+ ovarian, but it has stopped enrolling, and the only recruiting study (NCT06660511) is in China, limited to platinum-resistant relapse, and pairs the drug with anlotinib, whose bleeding and clotting risk runs

against her VTE and IVC-filter history. She is in first remission with no measurable disease, so neither study fits today, and the MMAE payload carries the peripheral neuropathy and neutropenia she has named as vetoes. Best held as a watch-list option for a future relapse.

Next steps

- 1. Call the Pfizer Clinical Trial Contact Center (1-800-887-7002) to ask whether NCT06003231 will reopen enrollment or a successor basket is planned; it reads active, not recruiting as of June 2026.
- 2. Treat NCT06660511 as China-only: the contact is Jingjing Wang (wangjingjing78@csu.edu.cn, 86+13574841167), but US enrollment is not available and the anlotinib combination clashes with her VTE/IVC-filter history.
- 3. Order a confirmatory HER2 re-stain; the basket accepts IHC 1+, but the single-specimen call is worth hardening before committing.
- 4. Revisit at relapse, since both trials require recurrent, measurable, platinum-resistant disease she does not have in first remission.
- 5. For manufacturer questions, RemeGen's line (4001110266) and pharmacovigilance inbox (remegenpv@remegen.com) are China-based; US development sits with Pfizer.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06003231		active not recruiting	no	Pfizer CT.gov Call Center; 1-800-887-7002
NCT06660511	Early Phase 1	recruiting	no	Jingjing Wang; wangjingjing78@csu.edu.cn; 86+13574841167

Manufacturer / sponsor contact

- **Company:** RemeGen Co., Ltd. (originator); ex-China development partnered to Pfizer via the former Seagen license
- **Med info phone:**4001110266
- **Med info email:**remegenpv@remegen.com
- **Product info:**<https://www.remegen.com/>
- **Notes:** 4001110266 (Mon-Fri 8:30-17:00 China time) and remegenpv@remegen.com are RemeGen's China-based product/pharmacovigilance contacts. For the US-run basket, route trial questions to the Pfizer Clinical Trial Contact Center (1-800-887-7002). No compassionate-use portal is published for the US.

Payer / coverage: Investigational in the US with no marketed product, so there is nothing to bill and no payer pathway. Within a trial the sponsor supplies the drug.

Notes: Status changed since the trial_screener pass: NCT06003231 is now active, not recruiting (registry last updated 2026-06-01), where the dossier recorded it as recruiting. Flagged so the user knows the US slot has closed.

9. GEN-1 / IMNN-001 (IL-12 immunogene, OVATION 2)

Aliases: GEN-1, IMNN-001, EGEN-001, IL-12 plasmid lipopolymer

Modality: other · **Regulatory:** Investigational; no FDA or EMA approval. IND-stage IL-12 DNA immunogene therapy (Imunon). FDA Fast Track designation reported for advanced ovarian cancer. · **Guidelines:** Not guideline-endorsed; investigational regimen the patient received en route to R0. · **Geographic scope:** US — only via sponsor-run trial sites; no routine access route. · **Verified:** 2026-06-24

GEN-1 / IMNN-001 has no approval anywhere, so the only route is a clinical trial, and the OVATION 2 study the patient was on has finished enrolling. Continued access depends on whether the sponsor's protocol or a successor study (Imunon has signaled a phase 3 confirmatory program) keeps her on therapy, or on a compassionate-use arrangement, neither of which is guaranteed. The first move is to confirm continuity directly with the treating site and the sponsor.

Next steps

1. Ask the treating trial site whether OVATION 2 has a continuation/extension provision keeping the patient on IMNN-001 after the primary phase.
2. Contact Imunon (1-609-896-9100, info@imunon.com) to ask about a planned phase 3 confirmatory trial and any expanded-access or compassionate-use pathway for a patient already responding to GEN-1.
3. If neither continuation nor an EAP is available, plan the maintenance decision around the approved options (niraparib, rucaparib, bevacizumab) instead.
4. Re-screen for a successor IMNN-001 trial once the phase 3 program registers on ClinicalTrials.gov.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03391284		completed	no	—

Manufacturer / sponsor contact

- **Company:** Imunon, Inc. (formerly Celsion Corporation)
- **Med info phone:** 1-609-896-9100
- **Med info email:** info@imunon.com
- **Product info:** <https://www.imunon.com/>
- **Compassionate use email:** info@imunon.com
- **Notes:** 1-609-896-9100 is Imunon's corporate / investor line (Lawrenceville, NJ); the company does not publish a dedicated medical-information number. info@imunon.com is the published general corporate inbox. No dedicated compassionate-use portal is published, so the corporate inbox is the verified contact for an extended-access inquiry; a phase 3 confirmatory program is what the sponsor has signaled rather than an EAP.

Payer / coverage: Investigational; the sponsor supplies the agent within the trial. No payer coverage applies, and there is no marketed product to bill.

Notes: Investigational-only. The intraperitoneal route also runs against the patient's stated oral/low-burden preference.

Unavailable (1)

10. Trastuzumab duocarmazine (SYD985)

Aliases: trastuzumab duocarmazine, SYD985, vic-trastuzumab duocarmazine

Modality: ADC · **Regulatory:** Discontinued. FDA issued a complete response letter in May 2023 for the HER2-positive metastatic breast filing; the EU marketing application (validated July 2022) never cleared; the global development status is now discontinued, with the trials completed or withdrawn. No approval anywhere. · **Guidelines:** Not guideline-listed; no approved indication. · **Geographic scope:** None; discontinued worldwide. · **Verified:** 2026-06-26

There is no way to get this drug. Byondis received an FDA complete response letter in May 2023, the EU application never cleared, and the program's global development status is now discontinued, with its trials completed or withdrawn. It never enrolled ovarian patients. It is listed here only so the record is explicit: this is not an option.

Next steps

1. No action available; the drug cannot be obtained, so record it as closed.
2. If a HER2 ADC is still the goal, the routes that exist are off-label Enhertu or a disitamab vedotin trial, both covered above.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02277717		completed	no	—

Manufacturer / sponsor contact

- **Company:** Byondis B.V.
- **Product info:**<https://www.byondis.com/>
- **Notes:** No marketed product and no compassionate-use pathway, since development is discontinued. No medical-information line is published for an approved product because there is none.

Payer / coverage: Not applicable; no marketed product.

Notes: Kept as a deliberate unavailable row so the user sees the closed door rather than a silent absence.